THE WHITE ARMY

Respiratory case

Ritwik Pattnaik
Final Year
MBBS

Patient Details

Name: Mr XYZ
Age: 64 years
Sex: Male
Address: Chandrasekharpur, Bhubaneswar
Occupation: Retired Lecturer
Socioeconomic status: Upper Class
Informant: Self
DOA: 24/9/2021
DOE: 25/9/2021

Chief Complaints

Cough X 5 days
Fever X 5 days
Breathlessness X 3 days

History of presenting illness

Cough X 5 days

-Sudden onset
-Gradually progressive
-Associated moderate mucopurulent yellow sputum
-Non foul smelling sputum
-No hemoptysis
-No postural or diurnal variation

Fever X 5 days

-High grade
-Associated with chills and sweating
-Continuous
-Not relieved with over the counter antipyretics
-No myalgia, joint pain or rash

Breathlessness X 3 days

-Gradual onset
-Gradually progressive
-Breathlessness at rest, MMRC grade 4
-No postural or diurnal variation
-No orthopnea or paroxysmal nocturnal dyspnea

-No history of nasal congestion, sore throat or wheezing
-No histrionics of vomiting, diarrhoea, abdominal pain or dysuria
-No history of weight loss, hoarseness of voice or dysphagia
-No history of chest pain, orthopnea, PND, palpitations, syncope, extremity edema or decreased urine output
-No history of altered mental sensorium.

Past History

-Diabetes mellitus X 10 years, on oral medication.
-No history of previous similar complaints, recurrent upper respiratory tract infection, asthma, COPD or previous treatment for Tuberculosis.
-No history of stroke, seizure, near drowning, recent surgery or dental procedure
-No history of hypertension or coronary artery disease
-No known allergies to drugs or environmental triggers.

Family History

-No history of similar complaints in family members
-No contact with patients of active tuberculosis
-No history of asthma,hay fever or eczema in family members.

Personal History

Diet: Balanced mixed diet
Appetite: Decreased since the onset of illness
Social: Retired lecturer
Monogamous relationship with his wife since last 40 years
Good living conditions, no pet, no recent travels
Addiction:10 cigarettes a day for 30 years
Whiskey on social occasions
Bowel and Bladder: Normal

Summary

A 64 year old diabetic male presented with complaints of cough with Expertorcation for 5 days. He has a significant smoking history of 15 pack years.

Differential diagnosis

Lower respiratory tract infection - pneumonia
Acute hypersensitivity pneumonitis
Infective endocarditis

General Physical Examination

-The patient is conscious, cooperative and well oriented to time, place and person.
-He is moderately built and moderately nourished.
Height: 175 cm
Weight: 70 kg
BMI: 22.9 kg/m2
-No external markers of tuberculosis
-No external markers of HIV.

-No pallor, icterus, clubbing, cyanosis, lymphadenopathy or edema

Vitals

Temperature: 101 F, oral

Pulse: 100 bpm, normal volume, regular rhythm, equally felt on both sides with no radioradial or radiofemoral delay. Condition of the vessel wall is normal. All peripheral pulses are palpable.

Blood Pressure: 100/70 mm hg measured in right upper arm in supine position

Respiratory rate: 25/min, Thoracoabdominal, accessory muscles active

O2 saturation: 94% on room air

JVP: Not elevated

Local Examination

Upper Respiratory Examination

The external appearance of the ear and auditory canal is normal
The nose appears normal. Septum is central and there are no secretions.
The posterior pharyngeal wall is mildly erythematous.
There is no sinus tenderness.

INSPECTION

-Trachea is central in location.
-Apex beat is not seen
-Chest shape is normal
-Chest movements are reduced on right side with the use of accessory muscles.
-Volume loss: There is no drooping of shoulders, crowding of ribs, supraclavicular or infraclavicular hollowing, subcoastal or intercostal retractions.
-Spine is cental with no kyphosis or scoliosis
-There are no scars, sinuses, visible pulsations or dilated veins over the skin

PALPATION

-No tenderness or crepitus
-Trachea is confirmed to be central.
-Apex beat is palpated at the left fifth intercoastal space on the mid clavicular line
-The AP diameter is 13 cm, horizontal diameter is 26 cm, AP to lateral ratio is 1:2. Chest circumference is 60 cm. Total chest expansion is 4 cm Expansion of right hemithorax is 1 cm, left hemithorax is 3 cm
-Vocal fremitus is increased over the right mammary and infra axillary areas.

PERCUSSION

-Percussion over the clavicle produced a reasonant note
-There is dullness on percussion over the right mammary and the right infra axillary areas compared to the corresponding lung areas of the left side.
-Other areas have reasonant nodes
-Upper border of the liver was percussed at the right 5th intercoastal space in the mid clavicular line
-Cardiac dullness was seen at the left of the sternum

AUSCULTATION

-Auscultation over the right mammary and infra axillary areas revealed bronchial breath sounds, increased vocal reasonance, aegiphony and whispering pectoriloquy.
-Coarse inspiratory crackles over the right infra axillary and infra scapular areas.
-Normal vesicular breath sounds are auscultated in other bilateral lung fields.
-Type and intensity: Bronchial breath sounds
-Adventitious sounds: Coarse inspiratory crackles
-Vocal reasonance: Increased, aegophony and whispering pectoriloquy present.

Systemic Examination

GI EXAMINATION

Soft non tender abdomen. No organomegaly. No ascites

Cardiovascular system

S1 and S2 normal. No murmurs

Nervous system

Higher mental functions are normal. No focal neurological deficits.

Provisional Diagnosis

My provisional diagnosis for this patient is community acquired pneumonia with a right middle to lower lobe consolidation and the patient is in respiratory distress due to type 2 respiratory failure.